

The Real-Cost Calculator

What a year of GLP-1 actually costs you — every path compared, no asterisks

The Real-Cost Calculator

This is a comparison tool, not a savings promise. It does not tell anyone what they *will* pay. It estimates what each path *could* cost based on what you enter. We compare **what each path costs**, not whether the medicines are the same.

What this is

Most "GLP-1 cost" pages show you one number with a footnote. The number people quote is rarely the number they pay. Copays change. Doses climb. Coverage lapses. This worksheet lets you enter your own situation and see a fair, honest, 12-month total for each path you could realistically take — side by side — so you can decide with eyes open.

What it compares (cost paths only)

1. **Brand-name GLP-1 + insurance** (copays, possible dose escalation, coverage gaps)
2. **Brand-name GLP-1 + cash / no coverage**
3. **Compounded GLP-1 membership** (flat monthly, clinician-managed)
4. **Doing nothing** (\$0, shown for honest context)

Step 1 — About you

Fill in your starting situation. These two answers shape the brand-name estimates, because branded GLP-1 cost often rises as the dose climbs.

Your situation	Your answer
What's your starting dose situation? (Just starting / Already on a low dose / Already on a higher or maintenance dose)	
Do you expect your dose to increase over the year? (Yes / No)	

Step 2 — Enter your numbers

Fill in what you know. If you don't know a number, leave it blank and use a labeled estimate (a sourced market range with a date). These are estimates, not quotes.

Path 1 — Brand-name + insurance

What to enter	Your number
Your monthly copay (if covered)	
Is it covered today? (Yes, covered / Need prior authorization / Not covered or denied / Not sure)	
Months you expect a coverage gap this year (0–12)	
Cash sticker price if you hit a gap	

A gap month = paying the cash sticker price, not your copay.

Path 2 — Brand-name + cash

What to enter	Your number
Cash price per month (no insurance)	

Path 3 — Compounded membership

What to enter	Your number
Flat monthly membership price	
One-time / onboarding fee (if any)	

Shared add-ons (applied to the medical paths)

What to enter	Your number
Visit / consult fees per year	
Labs or extras you expect to pay	

Step 3 — Your 12-month total, by path

Add up each path across a full year, then compare them side by side.

- **Brand-name + insurance:** (covered months × copay) + (gap months × cash sticker) + visit fees + labs
- **Brand-name + cash:** (12 × cash price) + visit fees + labs
- **Compounded membership:** (12 × flat monthly price) + onboarding fee + labs
- **Doing nothing:** \$0

Path	Your estimated 12-month cost	Notes
Brand-name + insurance		Climbs if dose increases or coverage gaps apply
Brand-name + cash		Climbs if dose increases
Compounded membership		Flat — doesn't climb with your dose
Doing nothing	\$0	Shown for honest context, not a recommendation

The membership total does not carry a dose-escalation increase: the dose can change under clinician management, while the monthly price stays flat. That is the structural difference this worksheet surfaces — not a promise that anyone saves a specific amount.

Honest difference between paths

Comparison	Estimated difference
Membership vs. brand-name + insurance	
Membership vs. brand-name + cash	

A difference is not a guarantee. If a path costs more for you, the honest result shows that too — never "you save \$X" or "guaranteed savings." A difference can be negative.

How to read your result

The honest read depends on which path lands lowest *for you* — not on a universal claim.

Pattern A — Membership is the lowest medical path (often when there's a coverage gap, dose escalation, or no coverage). For your situation, the flat monthly path totals less over 12 months than fighting copays and gaps. Honest, situational, not guaranteed.

Pattern B — Brand-name + insurance is the lowest (strong, stable coverage, low copay, no gaps). If your plan covers it well and reliably, that can be the cheapest path.

Pattern C — Paths are close. Cost isn't the deciding factor for you. Then the question becomes predictability (flat vs. variable), the insurance runaround, and whether you want a clinician managing dose changes without the bill changing.

The worksheet's job is clarity, not persuasion. If brand + insurance is cheaper for you, the honest result earns more trust than a rigged one.

Important — please read

These figures are **estimates**, not quotes. Prices vary by insurance plan, dose, pharmacy, region, and eligibility, and they change over time. This tool compares the **cost** of different paths — it does not compare medications or claim any product is the same as another. Treatment requires evaluation by a licensed clinician, and not everyone is eligible. Nothing here is medical advice or a promise of results. When a commercially available product isn't the right fit, a licensed clinician may determine a compounded option is appropriate for an individual patient; compounded medications are made by a licensed pharmacy and are not FDA-approved products.